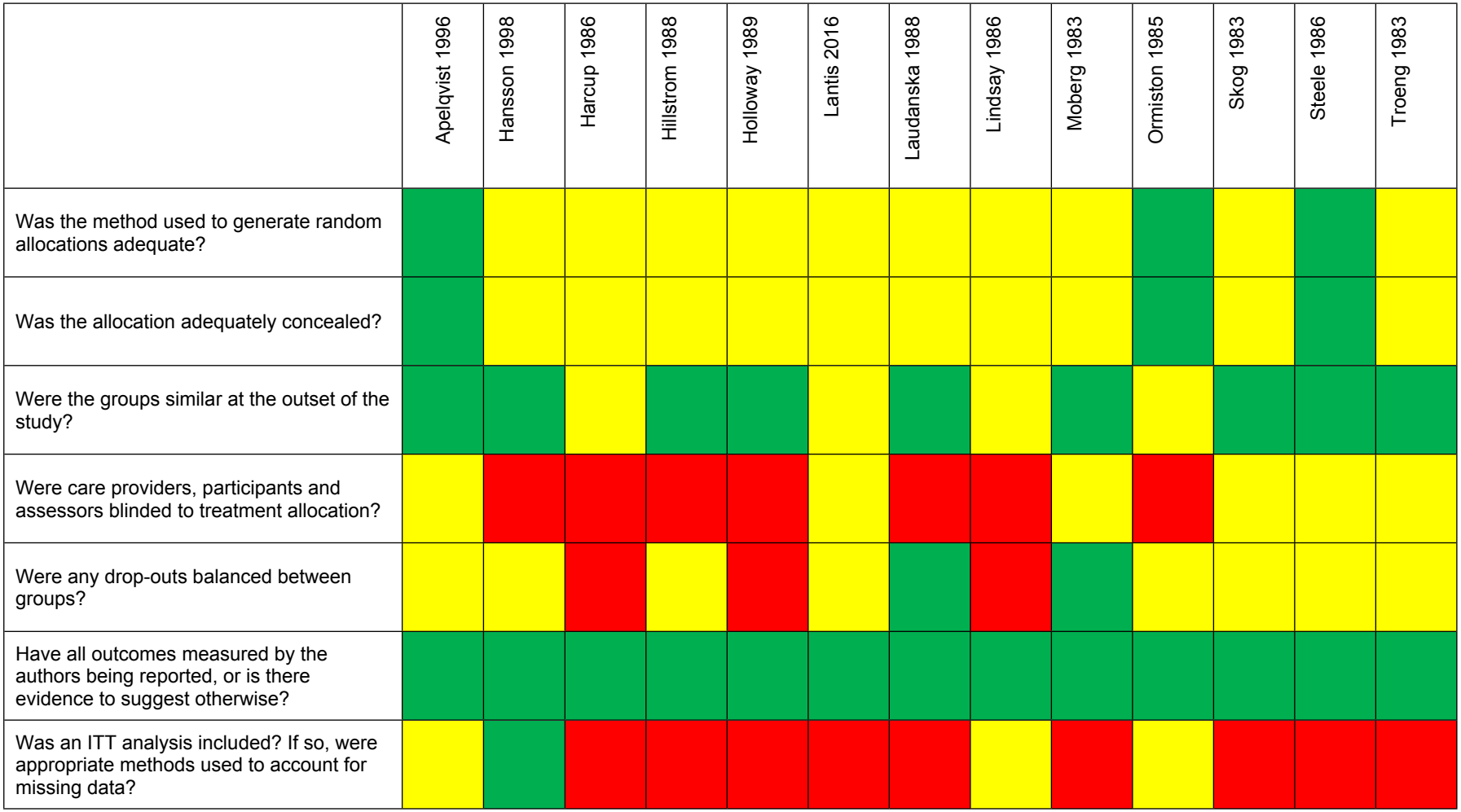
Supplement 1. Study quality and bias assessment

Quality criteria were taken from Centre for Reviews and Dissemination guidelines for the assessment of risk of bias in RCTs. Yellow indicates partial or unclear, red indicates missing criteria; ITT, intention to treat.



Supplement **2.** Summary of relevant studies for data extraction and outcomes with Cadexomer iodine (CIOD) identified in the systematic literature review, n=13 studies

VLU = Venous leg ulcer, DFU = Diabetic foot ulcer, PU = pressure ulcer/ injury, CIOD = Cadexomer iodine, SOC = standard care, NS = not significant

Study	Study design	Patient number/ indication	Outcomes of interest reported
Apelqvist 1996	RCT IODOSORB Ointment vs.SOC (gentamicin solution, streptodornase/strepto-kinase or dry saline gauze)	n=41 DFU	• Five patients treated with the CIOD were completely healed during the observation time (12 weeks), compared to 2 patients treated with standard dressings. • Average recorded number of weekly dressing changes was 4.7 in the CIOD group and 9.9 in the control group. • Significantly less total weekly costs in the CIOD group compared to SOC ($p < 0.01$)
Hansson 1998	RCT IODOSORB ointment vs. hydrocolloid dressing vs. paraffin gauze dressing	n=153 VLU	• Rate of healing (ulcer area reduction per week) was significantly higher for CIOD than for paraffin gauze ($0.64 cm^2$/week vs. $0.19 cm^2$/week, $p = 0.0353$). • Increased epithelialisation with CIOD compared to SOC over 12 week trial • Mean reduction in ulcer size in all patients was 62% with CIOD vs. 41 % and 24% for hydrocolloid and paraffin gauze (NS). • In patients treated for full 12 weeks (n=51) ulcer area reduction was 66% for CIOD and 18% for hydrocolloid.($p = 0.0127$). • 70% of the ulcers in the CIOD group ceased exudation during the study compared with 52% in the hydrocolloid group and 44 % in the paraffin gauze group. • CIOD treatment resulted in significantly less slough at 4 and 8 weeks than paraffin gauze.
Harcup 1986	Randomised optional crossover trial IODOSORB powder vs. SOC	n=72 VLU	• Significantly more patients healed after 8 weeks using CIOD – 13 in Cadexomer iodine group, 1 in SOC group ($p<0.005$) • CIOD demonstrated significant improvement compared to SOC by week 4 for ulcer size ($p < 0.01$), oedema ($p < 0.05$), erythema ($p < 0.05$), exudate ($p < 0.001$), odour ($p<0.01$), pus and debris ($p <0.05$) and pain ($p < 0.005$).

Study	Study design	Patient number/ indication	Outcomes of interest reported
Hillstrom 1988	RCT IODOSORB powder vs. Standard care	n=74 VLU	• Mean Ulcer size reduced significantly in week 1 following CIOD treatment and continued up to week 6 (end of trial) – by week 6 34% reduction (5 % increase SOC). • Patients that switched to CIOD from SOC – ulcer size gradually reduced. • Compared to standard care at 6 weeks, CIOD significantly stimulated granulation ($p < 0.01$), reduced erythema ($p < 0.001$) and reduced pus and debris ($p < 0.001$) • Significantly lower number of *Staphylococcus aureus* isolates CIOD treatment ($p < 0.001$) • Significant improvement in infection in 16 patients treated with CIOD compared to 0 in the SOC group.
Holloway 1989	Randomised crossover trial IODOSORB Powder vs. standard care	n=75 VLU	• Ulcers healed more than twice as rapidly using CIOD (n = 38) as with standard therapy (n = 37) ($p = 0.0025$). (healing rate 0.95cm^2/ week compared to 0.41cm^2/ week respectively) • Ulcers treated with CIOD showed trends towards less pain, exudate, pus, and debris, and a more rapid development of granulation tissue. • 12 patients crossed over from control treatment to the use of CIOD because of a failure to heal (no patients switched to control therapy) ($p = 0.01$).
Lantis 2016	RCT IODOSORB gel vs. SOLOSITE gel	n=14 DFU	• Biofilm counts were significantly reduced by 1.0 $\log_{10}$ cfu/g for CIOD versus an increase of 0.3 $\log_{10}$ cfu/g for hydrogel (95% CI for difference -0.3 to 4.5; $p=0.063$) within two weeks. • Reduction in biofilm-protected bacteria by four weeks was 1.0 $\log_{10}$ cfu/g for CIOD versus 0.8 $\log_{10}$ cfu/g for hydrogel (95% CI for difference -2.6 to 4.6; $p=0.337$). • Similarly, there appeared a general trend towards faster reductions in planktonic bacteria after treatment with CIOD compared to hydrogel.

Study	Study design	Patient number/ indication	Outcomes of interest reported
Laudanska 1988	RCT IODOSORB ointment vs. standard care	n=60 VLU	• Mean reduction in ulcer area after 6 weeks 71% in CIOD group and 54 % in SOC ($p < 0.001$ and $p < 0.01$ respectively compared to baseline) and significantly reduced compared to SOC ($p < 0.05$) • Significant difference in CIOD group compared to standard care over 6 weeks with more rapid reduction in pus and debris ($p < 0.01$), exudate ($p < 0.001$), erythema ($p < 0.05$), oedema ($p < 0.001$) and pain ($p < 0.001$) and significant increase in granulation ($p < 0.01$)
Lindsay 1986	RCT Multicentre optional crossover IODOSORB powder vs standard care	n=28 VLU	• Significant reduction in ulcer area using CIOD compared to SOC (33.6% vs. 4.2 % $p<0.005$) • Significantly improvement with CIOD compared to standard care in odour ($p<0.002$), pain ($p < 0.002$), Pus ($p < 0.002$), exudate ($p < 0.002$), erythema ($p < 0.005$), oedema ($p < 0.002$) • At 4 weeks 11/13 patients in standard care group changed to CIOD therapy (0 changed to the SOC group) • CIOD treatment resulted in decrease in organisms in most cases – associated to improvement in ulcer with a reduction in odour associated to elimination of coliforms and *Staphylococcus aureus.*
Moberg 1983	RCT IODOSORB powder vs. standard care	n=38 PU	• Significant reduction in ulcer area in 8/18 patients treated with CIOD compared to 1/18 in SOC ($p < 0.01$) and increased epithelialisation compared to SOC • 6 patients healed in CIOD group compared to 1 in the SOC group ($p < 0.05$) • IODOSORB superior compared to standard care in removing pus and debris ($p < 0.005$) and reducing pain ($p < 0.02$) by 8 weeks
Ormiston 1985	RCT Randomised optional crossover trial IODOSORB powder vs std. dressing (various dependant on local protocol)	n=61 VLU	• Significantly faster re-epithelialisation with CIOD compared to SOC ($p<0.001$) • Optional crossover at week 12 was in favour of CIOD ($p<0.02$) • Obvious improvement in granulation, oedema, exudate, pus and debris, and erythema observed after both treatments - trends favoured CIOD (NS).
Skog 1983	RCT IODOSORB powder vs std. dressing (various dependant on local protocol)	n=74 VLU	• Significant reduction in ulcer size with CIOD compared to SOC. (2 weeks $p<0.005$) • After 6 weeks of treatment the mean ulcer size in the CIOD group had reduced by 34%, and in the standard group it had increased by 5%

Study	Study design	Patient number/ indication	Outcomes of interest reported
			• CIOD was significantly more effective than standard treatment for reduction of pain ($p<0.01$), removal of pus and debris ($p<0.005$), removal of exudate ($p<0.005$), stimulation of granulation ($p<0.05$) and reduction of surrounding erythema ($p<0.005$) providing a marked debridement effect • CIOD significantly reduced infection with *Staphylococcus aureus, Pseudomonas aeruginosa* and other pathogenic organisms ($p<0.001$) • Correlation observed between the time taken to reduce/ eliminate in *S. aureus* and rate of healing. • CIOD was significantly more effective than the standard treatment against Beta-haemolytic streptococci, *Proteus*, Enterobacteria and *Klebsiella*.spp. ($p< 0.001$)
Steele 1986	RCT IODOSORB powder vs. standard treatment	n=57 VLUs	• At the end of the trial (6 weeks) three ulcers had completely healed in the CIOD group compared with one in the SOC group.
Troeng 1983	RCT IODOSORB powder vs. standard treatment (such as paraffin impregnated dressing)	n=74 VLUs	• Significant difference in ulcer size in favour of CIOD compared to SOC after 1 week ($p<0.01$), continued at each time point (week 2, 4 and 6; $p<0.005$, $p<0.01$, $p<0.02$ respectively) • Significant increase in granulation by 6 weeks with CIOD vs. SOC ($p<0.01$) • Significant reduction in pus and debris scores for CIOD at week 2 ($p<0.025$), week 4 ($p< 0.005$) and week 6 vs. SOC ($p<0.001$) • Ulcers became significantly less exudative after application of CIOD. • Significant reduction in *S. aureus* by 6 weeks vs. SOC ($p<0.01$) and also other organisms (including streptococci, Proteus, enterococci, enterobacteria and Klebsiella